C E R C A L A B S

AI Won’t Automate Your RCM Workflows — But These Tools Will

A Practical Guide for RCM Teams on Where AI Helps and Where Automation Does the Work

FREE GUIDE
RCM AUTOMATION
2026

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The Problem	The Framework	The Starting Point
Teams spend hours on manual RCM work and reach for the wrong tools to fix it.	AI for reasoning. Automation for execution. Two different tools for two different jobs.	Don’t start with a technology. Start with one workflow and map it first.

Executive Summary

Most RCM teams are not failing to adopt AI because they are resistant to technology. They are failing because nobody has shown them where it helps and more importantly, where it does not.

The result is one of two problems: teams avoid AI entirely because they assume HIPAA makes it off-limits, or they reach for consumer AI tools like ChatGPT to handle tasks those tools were never designed to execute. Either way, the manual work keeps piling up.

This guide draws a clear line between two tools that get conflated constantly:

- Generative AI — best used for reasoning, summarizing, drafting, and classifying
- Workflow automation — best used for repeatable task execution: pulling data, routing claims, assembling documents, updating systems

It also addresses a scenario that often gets overlooked: organizations that restrict or prohibit AI tools entirely. If that describes your environment, this guide will show you that automation, not AI, is where most of the real efficiency gains live anyway.

> **The core principle**
> Start with one workflow, not one technology. Map it, measure it, separate execution from judgment, and automate the repeatable pieces first. Then layer in AI where it actually helps.

Section 1: The Problem Most RCM Teams Don't Name

Walk into most RCM departments and you will find the same scene: billing coordinators logged into their respective billing systems, downloading claim lists into Excel, manually filtering by denial reason codes, copying data between systems, and processing each item one by one.

This is not a technology problem. It is a workflow problem that technology has not yet been applied to.

Here is what a typical appeals workflow looks like before and after automation:

Before Automation	After Automation
Log into billing system and download appeal list	System automatically identifies and routes denied claims by reason code
Open Excel, manually filter by denial reason codes	Claims are pre-filtered and queued without manual intervention
Review each claim individually to determine next action	Automation pulls required documents from the relevant source systems
Manually pull supporting documents from multiple systems	Appeal package is assembled and formatted automatically
Assemble appeal package by hand for each claim	Appeals submitted daily on a consistent automated schedule
Submit appeal to payer portal manually	Team reviews completed packages as QA, not as assemblers
Track submission status manually	Team focuses on identifying revenue gaps and high-value exceptions

The same pattern applies to refund processing, eligibility error review, prior auth status follow-up, and claim rejection handling. In each case, a billing team member is acting as the filter between a system and a decision, and that filter step is almost entirely automatable.

At one large reference lab, automating these workflows produced:

- 80,000+ RCM transactions automated per month
- 10,000-claim appeal backlog cleared through automated document retrieval and package assembly
- Appeal processing time reduced from 5–10 minutes per package to roughly 2.5 minutes
- 1,300+ appeals processed weekly, current with zero backlog

→ Eligibility error visibility built before claims leave the building

The team did not need to get larger. The workflow got smarter. Billing coordinators shifted from processing claims to reviewing them and from executing repetitive tasks to identifying the revenue gaps worth pursuing.

> **The shift that matters**
> When automation handles execution, the billing team stops being data entry operators and starts being revenue analysts. That is the real value of getting this right.

Section 2: The HIPAA Confusion Holding Teams Back

Ask most RCM teams why they have not adopted AI tools and the answer is usually some version of: “We were not sure if it was allowed under HIPAA.”

That hesitation is understandable. But it is often based on a misread of where the actual risk lives.

> **Note**
> This is not legal or compliance advice, but it is a practical operating distinction RCM teams can use when evaluating AI use cases. Always consult your compliance and legal teams before implementing AI tools in your workflows.

The real line is not AI vs. no AI

HIPAA does not block AI by default. The risk starts when patient-identifiable information enters a tool or environment that is not approved to receive it, specifically one that lacks the right security controls, internal approval, and in most cases a Business Associate Agreement with the vendor.

HHS is clear that cloud and service providers handling ePHI on behalf of a covered entity need a HIPAA-compliant BAA and must meet HIPAA requirements. The tool category matters less than the data environment. What matters is whether PHI is entering a governed, controlled environment.

What is generally safe

There is a large category of AI use cases that do not involve PHI and generally carry lower compliance risk:

- Writing or improving denial appeal templates
- Drafting workflow documentation and process guides
- Summarizing payer rules or coverage policies
- Generating Excel formulas or SQL for internal reporting
- Creating training materials for billing staff
- Analyzing de-identified trend data

> **Practical example**
> “Write a denial appeal template for timely filing” = safe. “Here is a patient’s claim history and denial letter, help me write an appeal” = not safe in a consumer tool.

What to avoid

Do not paste the following into consumer AI tools unless covered by an enterprise agreement with HIPAA-compliant terms and internal approval:

- Patient names, dates of birth, or MRNs
- Insurance IDs or claim numbers tied to a specific patient
- Clinical notes, lab reports, or medical records
- EOBs or denial letters
- Screenshots from billing systems containing patient data

Where enterprise AI may be acceptable

If an AI tool is approved by the organization, covered by appropriate vendor terms and a BAA, configured not to train on submitted data, access-controlled, and reviewed by compliance and security teams, then PHI use may be appropriate. But that is a governed deployment, not casual staff-level copy and paste.

For most RCM teams the practical answer is straightforward: use AI for work that does not involve patient data, and use workflow automation tools, which operate entirely inside your existing infrastructure, for the task execution that does.

Section 3: What If Your Organization Restricts AI Tools Entirely?

Some organizations have policies that restrict or prohibit the use of generative AI tools altogether. If that describes your environment, this section is for you, and the message is straightforward: you are not as far behind as you might think.

RCM automation did not start with AI

Revenue cycle automation has existed for decades. Long before ChatGPT, RCM teams were using robotic process automation, scripting, API integrations, and rules-based workflow tools to handle high-volume, repetitive billing work. Denial routing, claim scrubbing, eligibility verification, prior auth tracking, document retrieval, all of this was being automated with tools that have nothing to do with generative AI.

Generative AI is a recent addition to the toolkit. It is not the foundation of the toolkit.

> **The important distinction**
> AI and automation are not the same thing. Many organizations, and even many RCM professionals, use the terms interchangeably, but they describe fundamentally different technologies. Automation executes defined, repeatable processes. AI generates outputs based on patterns in data. One does not require the other.

What you can still accomplish without AI

If your organization restricts AI tools, you can still automate the majority of your highest-value RCM workflows without touching a single AI model. The automation layer, RPA, scripting, API integrations, and workflow platforms, does not require AI and operates entirely within your existing infrastructure and data controls.

Workflows that are fully automatable without AI:

- Denial identification and routing by reason code
- Appeal package assembly and document retrieval
- Claim rejection pulling and resubmission queuing
- Eligibility error identification before submission
- Prior auth status tracking and follow-up
- Refund identification and processing queues
- Payer portal status checks and updates
- Automated daily submission of appeal packages

AI restrictions are not the end of the road

Organizations that restrict AI tools are often making a reasonable risk management decision. The compliance landscape around AI in healthcare is still evolving, and caution is understandable. But that caution should not prevent investment in workflow automation, which carries a fundamentally different risk profile.

The practical path forward for AI-restricted environments:

1. Audit your highest-volume manual workflows using the framework in Section 5
2. Identify which steps are rules-based and repeatable
3. Build the automation layer using RPA, scripting, or API integrations
4. Measure the results and build a track record
5. Revisit AI use cases later, when the governance framework is clearer and your automation foundation is already in place

> **The bottom line for AI-restricted organizations**
>
> You do not need AI to automate your RCM workflows. The tools that eliminate manual billing work, RPA, scripting, workflow automation, API integrations, have been proven in healthcare operations for years. Start there. AI can come later.

Section 4: Two Tools, Two Jobs

The core mistake most RCM teams make is treating generative AI and workflow automation as the same category of tool. They are not. They solve different problems and belong in different parts of the operation.

Generative AI *Reasons, summarizes, drafts, classifies*	**Workflow Automation** *Executes, routes, assembles, submits*
Drafting appeal language for human review	Pulling denied claims from the billing system automatically
Summarizing clinical notes for auth context	Routing claims by denial reason code without manual filtering
Identifying denial patterns across large datasets	Assembling appeal packages from multiple source systems
Explaining payer rules in plain language	Submitting appeals to payer portals on a daily automated schedule
Generating report narratives	Updating claim status in billing system after payer response
Writing workflow documentation	Triggering eligibility checks before claims are submitted
Answering staff questions about billing policies	Following up on prior auth status via payer portal APIs

> **The two-layer rule**
> If the task is repeatable, rules-based, and needs to happen the same way every time, automate it with workflow tools such as scripting, RPA, or APIs. If the task requires summarizing, drafting, classifying, or analyzing, AI may help, but only inside the right data-governed environment.

One belongs inside your organization's infrastructure where PHI is controlled and the process is auditable. The other belongs in a governed environment where outputs are reviewed before they act on anything. Conflating the two creates both operational failure and compliance exposure.

Section 5: Where to Start — The Workflow Audit

"We need AI" is not a project. "We need to reduce the time our team spends manually assembling appeal packages" is a project.

The first real step is a workflow audit. Pick one high-volume, repetitive workflow and map what happens from start to finish before choosing any tool.

Step 1: Pick the right workflow to start with

Choose a workflow that is high-volume, repetitive, and rules based. Good candidates in most RCM environments:

- Denial routing and appeal package assembly
- Eligibility error identification and correction
- Prior auth status follow-up
- Claim rejection handling and resubmission
- Refund identification and processing
- Payer portal status checks
- Missing documentation follow-up

Step 2: Map the workflow completely

For the workflow you select, document every step using these seven questions:

- What triggers the work?
- What systems does the team touch?
- What data is copied, checked, or moved between systems?
- Which steps are purely rules-based?
- Which steps require human judgment?
- Where does PHI appear in the process?
- What output needs to happen at the end?

Step 3: Separate execution from judgment

Once the workflow is mapped, categorize each step using this framework:

Step type	Belongs in	Example
Repeatable, rules-based	Workflow automation (RPA, scripting, APIs)	Routing denied claims by reason code
Data retrieval or system update	Workflow automation	Pulling documents, updating claim status
Drafting or summarizing	AI (in non-PHI or governed context)	Writing appeal language from a template
Pattern recognition across data	AI analytics (governed)	Identifying denial trends by payer
Complex judgment call	Human review	Determining clinical necessity for an appeal

A real example: the appeal workflow audit

If appeals are the bottleneck, do not start by asking AI to "do appeals." Start by mapping the appeal workflow specifically:

- How are denied claims identified and where are they stored?
- Where are denial reasons recorded and in what format?
- Which documents are required for each denial type?
- Where are those documents pulled from?
- Which appeal templates are used for which denial categories?
- How is the package assembled and in what format?
- Where and how is it submitted?
- How is the outcome tracked?

When you map this honestly, you often find that most of the time savings come from document retrieval, claim routing, package assembly, and status tracking, not from generative AI at all. That is the part most teams miss.

> **The insight**
>
> AI can be useful in RCM, but only after the workflow is understood. If the process is unclear, AI just makes the confusion faster.

Section 6: Implementation Checklist

Use this checklist to move from workflow audit to your first automation. Work through it in order.

Phase 1: Identify and map

- Select one high-volume, repetitive workflow as the starting point
- Map every step from trigger to output using the seven questions in Section 5
- Identify where PHI appears and which steps involve patient data
- Measure the current baseline: time, frequency, and staff hours per week
- Mark each step as: rules-based execution, judgment-required, or AI-appropriate

Phase 2: Design the automation layer

- Identify which systems need to be connected (billing platform, payer portals, document storage, clearinghouse)
- Confirm API availability or determine if RPA tooling is needed for systems without APIs
- Define the trigger that starts the workflow and the output that ends it
- Design exception handling: what happens when the automation encounters something it cannot process
- Confirm the data stays within your organization's controlled infrastructure throughout

Phase 3: Build, test, and deploy

- Build the automation on a subset of real volume first
- Run parallel processing: automation and manual side by side to validate accuracy
- Define the QA role for the billing team during and after transition
- Document what the team should review, flag, and escalate
- Deploy to full volume once parallel results are consistent

Phase 4: Layer in AI where it helps (if applicable)

- Identify the judgment and drafting tasks that remain after automation handles execution
- Confirm whether your organization permits AI tool use and under what conditions
- Start with non-PHI AI use cases first: template drafting, documentation, training materials
- For PHI-adjacent AI use, work with compliance and IT to identify approved tools and BAA requirements
- Measure AI output quality and keep human review in the loop until accuracy is confirmed

The right sequence

Map the workflow. Automate the repeatable execution. Measure the results. Layer in AI for the reasoning and drafting tasks that remain, if and when your organization is ready for it. Each phase builds on the one before it.

Conclusion

The RCM teams making real progress with automation are not the ones who picked the most sophisticated tool. They are the ones who started with the most honest question: where is our team spending time on work that should not require a human?

The answer is almost always a workflow problem, not a technology gap. Getting the repeatable execution out of human hands first and then layering in AI for the reasoning and drafting work that genuinely benefits from it, is the sequence that produces real results.

HIPAA is not usually the real obstacle. Unclear workflows are. And for organizations that restrict AI tools entirely, the automation layer alone is enough to produce meaningful efficiency gains, because most of the manual work in RCM was never an AI problem to begin with.

Once a workflow is mapped, the questions become specific and answerable: which steps involve PHI, which tools are approved to handle it, and which tasks can be handled outside that boundary. That clarity is worth more than any technology decision made before the workflow is understood.

Start with one workflow, not one technology
The teams seeing real efficiency gains did not start with an AI pilot. They started by mapping one painful workflow and automating the cleanest repeatable piece first. The tool followed the problem, not the other way around.

About the Author

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Jeff specializes in RCM automation and workflow strategy for specialty and genomics labs. His work focuses on building automation systems on top of the platforms labs already use — including Telcor, Waystar, UiPath, and ABBYY, so teams can improve operations without replacing their core systems.

If your team is dealing with high claim volume, manual RCM workflows, or questions about where automation fits, feel free to reach out. **linkedin.com/in/jeffreyafana** · **cercalabs.com**